

Clinical Presentation of Actinic Keratosis (AK)

Common signs and symptoms of actinic keratosis include **pruritus, burning or stinging pain, bleeding, and crusting**. The typical AK lesion, often referred to as the **erythematous AK**, most commonly appears as a 2- to 6-mm **erythematous, flat, rough, or scaly papule**. These lesions are often **more easily felt than seen** due to their texture. AKs can vary in size and may grow to several centimeters in diameter.

They typically arise on areas of **chronic sun exposure** and are found against a background of **photodamaged skin (dermatoheliosis)**, characterized by: - **Solar elastosis** - **Dyspigmentation** - **Yellowish discoloration** - **Ephelides (freckles)** - **Telangiectases** - **Sagging skin**

In some cases, the number and coalescence of AKs are so extensive that they resemble a **diffuse rash**.

Clinical Subtypes of Actinic Keratosis

In addition to the classic erythematous AK, several clinical variants exist:

Hypertrophic Actinic Keratosis (HAK)

- Presents as a **thicker, scaly, rough papule or plaque**
- Color may be **skin-colored, gray, or erythematous**
- Commonly located on **dorsal hands, arms, and scalp**
- May develop from a typical erythematous AK
- Can be clinically indistinguishable from **squamous cell carcinoma (SCC)**
- **Induration, increased thickness, pain, or ulceration** may indicate transformation to SCC, warranting **biopsy**

Cutaneous Horn (Cornu Cutaneum)

- A **reaction pattern**, not a specific diagnosis
- Appears as a **conical, hyperkeratotic protuberance** arising from a **skin-colored to erythematous papular base**
- By definition, the **height is at least half the largest base diameter**
- **38% to 40%** of cutaneous horns are associated with **AKs**
- Histopathologic examination may reveal various underlying lesions, including:
 - **Actinic keratosis**
 - **Squamous cell carcinoma**
 - **Seborrheic keratosis**
 - **Verruca vulgaris (common wart)**
 - **Trichilemmoma**
 - **Keratoacanthoma**
- In **elderly, fair-skinned individuals**, the most common underlying lesion is **hypertrophic actinic keratosis**

Actinic Cheilitis

- Represents **confluent AKs on the lips**, most commonly affecting the **lower lip**
- Clinical features include:
 - **Erythema**
 - **Scaling**
 - **Chapping**
 - **Fissures or erosions**
 - **Indistinct vermilion border**
 - **Focal hyperkeratosis**
 - **Leukoplakia**
- Patients often report **chronic dryness, cracking, and discomfort**
- Should be suspected in **elderly patients** with persistent cheilitis

Note: The presence of actinic cheilitis indicates significant cumulative sun damage and carries an increased risk of progression to SCC.